

CLOBESOL CREAM 0.05% W/W

NAME AND STRENGTH OF ACTIVE SUBSTANCE:

Each gram contains

Clobetasol Propionate.....0.5mg

PRODUCT DESCRIPTION:

A white cream base in printed aluminium tubes with white caps.

PHARMACODYNAMICS:

Topical corticosteroids act as anti-inflammatory agents via multiple mechanisms to inhibit late phase allergic reactions including decreasing the density of mast cells, decreasing chemotaxis and activation of eosinophils, decreasing cytokine production by lymphocytes, monocytes, mast cells and eosinophils, and inhibiting the metabolism of arachidonic acid.

Topical corticosteroids, have anti-inflammatory, antipruritic, and vasoconstrictive properties.

PHARMACOKINETICS:

Absorption

Topical corticosteroids can be systemically absorbed from intact healthy skin. The extent of percutaneous absorption of topical corticosteroids is determined by many factors, including the vehicle and the integrity of the epidermal barrier. Occlusion, inflammation and/or other disease processes in the skin may also increase percutaneous absorption.

Distribution

The use of pharmacodynamic endpoints for assessing the systemic exposure of topical corticosteroids is necessary due to the fact that circulating levels are well below the level of detection.

Metabolism

Once absorbed through the skin, topical corticosteroids are handled through pharmacokinetic pathways similar to systemically administered corticosteroids. They are metabolised, primarily in the liver.

Elimination

Topical corticosteroids are excreted by the kidneys. In addition, some corticosteroids and their metabolites are also excreted in the bile.

INDICATION:

Clobetasol propionate is a very active topical corticosteroid which is of particular value when used in short courses for the treatment of more resistant dermatoses such as psoriasis (excluding widespread plaque psoriasis), recalcitrant eczemas lichen planus, discoid lupus erythematosus, and other conditions which do not respond satisfactorily to less active steroids.

RECOMMENDED DOSAGE:

Apply sparingly to the affected area once or twice daily. Therapy should be discontinued when control is achieved. Treatment should not be continued for more than four weeks without the patient's condition being reviewed. Repeated short courses of CLOBESOL CREAM may be used to control exacerbations. If continuous steroid treatment is necessary, a less potent preparation should be used.

In very resistant lesions, especially where there is hyperkeratosis, the anti-inflammatory effect of CLOBESOL CREAM can be enhanced, if necessary, by occluding the treatment with polythene film.

Overnight occlusion only is usually adequate to bring about a satisfactory response. Thereafter improvement can usually be maintained by application without occlusion.

ROUTE OF ADMINISTRATION:

External, locally applied

CONTRAINDICATIONS:

Hypersensitivity to the active substance or any of the excipient.

The following conditions should not be treated with CLOBESOL CREAM:

- Untreated cutaneous infections
- Rosacea
- Acne vulgaris
- Pruritus without inflammation
- Perianal and genital pruritus
- Perioral dermatitis

Clobetasol is contraindicated in dermatoses in children under one year of age, including dermatitis and nappy eruptions.

WARNINGS AND PRECAUTIONS:

Long term continuous therapy should be avoided where possible, particularly in infants and children, as adrenal suppression can occur even without occlusion.

It is recommended that the treatment should be reviewed weekly if Clobesol Cream 0.05% w/w is required for use in children.

It should be noted that the infant's nappy may act as an occlusive dressing.

If used in childhood or on the face, courses should be limited if possible to five days and occlusion should not be used.

The face, more than other areas of the body, may exhibit atrophic changes after prolonged treatment with potent topical corticosteroids. This must be borne in mind when treating such conditions as psoriasis, discoid lupus erythematosus and severe eczema.

If applied to the eyelids, care is needed to ensure that the preparation does not enter the eyes, as glaucoma might occur. If Clobesol Cream 0.05% w/w does enter the eye, the affected eye should be bathed in copious amounts of water.

Topical steroids may be hazardous in psoriasis for a number of reasons including rebound relapses, risk of generalised pustular psoriasis, development of tolerance and development of local or systemic toxicity due to impaired barrier function of the skin. If used in psoriasis careful patient supervision is important.

When treating inflammatory lesions which have become infected, appropriate antimicrobial therapy should be used. Any spread of infection requires withdrawal of topical corticosteroid therapy and systemic administration of antimicrobial agents. Occlusive dressings induce warm moist conditions and encourage bacterial infection. The skin should be cleansed before a fresh dressing is applied.

There have been a few reports in the literature of the development of cataracts in patients who have been using corticosteroids for prolonged periods of time. Although it is not possible to rule out systemic corticosteroids as a known factor, prescribers should be aware of the possible role of corticosteroids in cataract development.

Clobesol Cream 0.05% w/w contains cetostearyl alcohol which can cause local skin reactions (eg. Contact dermatitis) and propylene glycol which may cause skin irritation.

INTERACTIONS WITH OTHER MEDICAMENTS:

Unknown.

PREGNANCY AND LACTATION:

There is inadequate evidence of safety in human pregnancy. Topical administration of corticosteroids to pregnant animals can cause abnormalities of fetal development including cleft palate and intrauterine growth retardation. The relevance of this finding to humans has not been established, therefore, topical steroids should not be used extensively in pregnancy, i.e. in large amounts or for prolonged periods.

The safe use of clobetasol propionate during lactation has not been established.

SIDE EFFECTS:

Immune system disorders

Hypersensitivity

Local hypersensitivity reactions such as erythema, rash, pruritus, urticaria and allergic contact dermatitis may occur at the site of application and may resemble symptoms of the condition under treatment.

If signs of hypersensitivity appear, application should be stopped immediately.

Endocrine disorders

Features of Cushing's syndrome

As with other topical corticosteroids, prolonged use of large amounts, or treatment of extensive areas can result in sufficient systemic absorption to produce the features of Cushing's syndrome. This effect is more likely to occur in infants and children, and if occlusive dressings are used. In infants, the nappy may act as an occlusive dressing.

Provided the weekly dosage is less than 50g in adults, any suppression of the HPA axis is likely to be transient with a rapid return to normal values once the short course of steroid therapy has ceased. The same applies to children given proportionate dosage.

Vascular disorders

Dilatation of the superficial blood vessels

Prolonged and intensive treatment with highly-active corticosteroid preparations may cause dilatation of the superficial blood vessels, particularly when occlusive dressings are used, or when skin folds are involved.

Skin and subcutaneous tissue disorders

Local skin burning, local atrophy, striae, thinning, pigmentation changes, hypertrichosis, exacerbation of underlying symptoms, pustular psoriasis.

Prolonged and intensive treatment with highly-active corticosteroid preparations may cause local atrophic changes, such as thinning and striae.

Treatment of psoriasis with corticosteroids (or its withdrawal) is thought to have provoked the pustular form of the disease.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Symptoms

Topically applied clobetasol may be absorbed in sufficient amounts to produce systemic effects. Acute overdosage is very unlikely to occur, however, in the case of chronic overdosage or misuse the features of hypercortisolism may occur.

Management

In the event of overdose, clobetasol should be withdrawn gradually by reducing the frequency of application or by substituting a less potent corticosteroid because of the risk of glucocorticosteroid insufficiency.

Further management should be as clinically indicated or as recommended by the national poisons centre, where available.

EFFECT ON ABILITY TO DRIVE AND USE MACHINE

There have been no studies to investigate the effect of clobetasol on driving performance or the ability to operate machinery. A detrimental effect on such activities would not be anticipated from the adverse reaction profile of topical clobetasol.

INSTRUCTION FOR USE

For external use only.

DOSAGE FORMS AND PACKAGING AVAILABLE:

Packing Size : 1x15g (aluminium tube), 50x15g (aluminium tubes)

STORAGE CONDITION

Store in the original package and in a dry place. Do not store above 30°C. Protect from light.

Keep out of reach of children.

NAME AND ADDRESS OF MANUFACTURER/PRODUCT HOLDER REGISTRATION

MALAYSIAN PHARMACEUTICAL INDUSTRIES SDN BHD (101323-U)

Plot 14, Lebuhraya Kampung Jawa, 11900 Bayan Lepas,

Pulau Pinang, Malaysia.

DATE OF REVISION:

02/03/2018